

KING FAHD CENTRAL HOSPITAL (KFCH)

Comprehensive Stroke Program

MONTHLY STROKE CODE AUDIT TOOL

Document ID	SU-009-AUD-01	Version	1.0	Audit Month	
Auditor		Department		Audit Date	

Purpose: Monthly review of Stroke Code cases to verify pathway compliance, time-critical performance, documentation quality, reperfusion/transfer processes, and closure of identified variances. Use de-identified case identifiers in accreditation evidence copies.

A. Case-Level Audit

Case ID	Arrival	EMS pre-notification	Stroke Code	LKW documented	NIHSS	Glucose	CT start	CT result/interpretation	CTA/LVO assessment	IVT eligible / given	DTN	MT eligible / transfer	DTP/groin*	Dysphagia	Disposition	Variance / action

* DTP/groin: document the locally applicable thrombectomy metric (e.g., door-to-puncture/groin or transfer timing) according to the current KFCH KPI definition. Do not mix different definitions in the same monthly trend.

B. Monthly Performance Summary

Indicator	Numerator	Denominator	Result	Target	Status / Comment
Total Stroke Code activations					
EMS pre-notification documented					
LKW documented					
NIHSS documented					
Door-to-CT compliance					
CT interpretation/imaging TAT compliance					
IV thrombolysis cases					
Door-to-needle compliance					
LVO/CTA assessment compliance					
Mechanical thrombectomy cases					
MT/transfer time compliance					
Dysphagia screening compliance					
Cases with documented variance review					



C. Variance, Root Cause & Corrective Action

Case / Issue	Variance	Root Cause	Corrective Action	Owner	Due Date	Closure Evidence / Date

D. Governance Review & Sign-Off

Reviewed at Stroke Committee / IPC	☐ Yes ☐ No	Meeting date / reference	_____
Escalation to Quality / Leadership required	☐ Yes ☐ No	Reference	_____
Stroke Coordinator	_____	Date	_____
Stroke Medical Director / Designee	_____	Date	_____

Document linkage: Use with KFCH Stroke Code Activation Log, Stroke Alert (Code Stroke) Activation Policy, Stroke System of Care, Stroke KPI Dashboard, Stroke Quality Improvement Program, and Annual Stroke Program Evaluation. Retain supporting de-identified case evidence for identified variances and improvement actions.